# LexisNexis® Risk Solutions Proposal for Centers for Medicare & Medicaid Services [CMS-0042-NC] RIN 0938-AV68; Request for Information; Health Technology Ecosystem

**June 16, 2025**

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June 16, 2025

Secretary Robert F. Kennedy, Jr.
Centers for Medicare & Medicaid Services,
Department of Health and Human Services,
Attention: CMS-0042-NC,
Mail Stop C4-26-05,
7500 Security Boulevard,
Baltimore, MD 21244-1850

Re: Request for Information; Health Technology Ecosystem

Dear Secretary Kennedy:

LexisNexis® Risk Solutions (LNRS) appreciates the opportunity to provide this response to the Centers for Medicare & Medicaid Services' (CMS) Request for Information for a Health Technology Ecosystem.

As the global leader in identity data management and the issuance of vital records, LNRS is uniquely positioned to offer insight into the role of comprehensive identity assurance in advancing CMS' goals across the healthcare ecosystem. Equipped with industry-leading data assets, advanced analytics, and proven capabilities in identity verification and referential linking for over 40 years, LNRS empowers government agencies and private sector partners to make informed decisions and mitigate risk in complex environments. Our innovative approach to digital enablement can support CMS in expanding access to trusted information, protecting privacy, and driving digital transformation.

We are eager to share ways in which CMS can encourage and benefit from the promotion of digital credentials to expand access to trusted information, empower informed health decisions, and increase data availability across the healthcare ecosystem. By promoting the adoption of interoperable digital credentials, CMS can place individuals in control of their own records, enable better-informed health decisions, and increase data availability for the diverse stakeholders working to improve health outcomes.

We look forward to continued engagement and discussions on these topics and are prepared to support follow up discussions including proof of concepts and demonstrations of our capabilities to assist CMS in designing a solution that addresses the significant opportunity to reduce administrative burden, accelerate interoperability and ultimately improve transparency and access to care. We stand ready to answer specific questions, discuss potential solution options and share how many of these solutions are efforts underway with many other agencies.

Sincerely,

Patricia Mangiacotti
Capture Manager
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# Overview

## An Identity-Informed Healthcare Ecosystem

LNRS is a global leader in identity verification and risk analytics, using a vast network of data sources to validate identities and worldwide. Our technology ensures secure, real-time identity proofing with minimal user friction, backed by advanced data analytics and global intelligence. Our technology is ubiquitous among the global fintech community, and industry experts like Gartner® and Forrester® have long documented our leadership in this space.

We are not only the leading identity data vendor – we are also the nation's most trusted issuer of birth and death records, working directly with citizens and state records offices. **With this combination of core capabilities, we are uniquely poised to provide commentary regarding identity management and the identity assurance lifecycle as it relates to the healthcare ecosystem and the goals expressed in CMS' RFI.**

With this strong foundation of 40+ years in managing identity data, our approach to maintaining our leadership advantage in this market is based on proactive planning on a 10+ year window into the future. The commentary we provide here is based on that planning timeframe – presenting the implications of moving to full-scale, patient-owned digital credentials within the American healthcare marketplace

## Identity Assurance, Security, and Digital Credential Management

Digital identity is a rapidly advancing market space and the best practices that were put in place near a decade ago are no longer industry leading. As a result, **we recommend that CMS evaluate the acceptance of Government-issued verifiable credentials bound by best practice identity assurance workflows over legacy CSP approaches.**

**The Evolution:** Initially, agencies implemented identity verification (IDV) solutions to address specific needs and mitigate known risks. While these practices are still utilized across workflows today, the market has shifted to federated identity approaches enacted by CSPs – a verify once, trusted by many approach that is highly centralized. This federated approach has numerous challenges that range from privacy and security concerns due to centralized data with inconsistent levels of trust used to establish these credentials, to intense user and agency friction – to name but a few. These challenges ultimately resulted in a pivot to the next generation of digital identity, one already underway and far more privacy preserving – the shift to government issued, decentralized, privacy preserving verifiable credentials.

These credentials, issued by a trusted government agency, live within holders' wallets, are not stored in centralized systems, and provide the holder the flexibility to decide what information they want to

present – where, how, and when. LNRS supports multiple government partners on this next wave of privacy preserving digital identity migration. As CMS evaluates government-issued verifiable credentials, our recommendation is look broadly across the wide variety of verifiable credential types that can be provided by government agencies. Mobile Driver's Licenses (mDLs), an early form of verifiable credential in the United States, are but one of many potential government-issued sources of verifiable credentials in the United States that CMS should consider as part of their acceptance processes. CMS is also in a unique position to both be a leading verifier of government-issued verifiable credentials as well as an issuer of these credentials for multiple entities within the healthcare ecosystem. By taking on these roles, CMS can improve both patient and provider experiences as it relates to government, business, and citizen/consumer interactions.

## *Benefits of Government-Issued Verifiable Credentials:*

- **Identity Assurance:** Digital identity, when done right, can establish high assurance trust across digital interactions. However, to achieve this goal, agencies must adhere to high assurance standards both upfront and ongoing to ensure that the identity being proofed and utilized to establish a verifiable credential is correct at the point of creation as well as over time.

  As part of the initial vetting, agencies should, at minimum, require a NIST IAL2 process, but even this approach is dated. As an issuer, LNRS recommends CMS consider a comprehensive assessment including one that evaluates the digital intelligence of the device, the risks associated with a user's email and phone, that leverages consortium insights of industry identified fraud/risks, assess the behavioral intelligence of the identity and the device in question, undergoes robust document authentication, biometric, and liveness using active capture and active fraud signaling via movie clips over static images, as well as assessing the physical intelligence of the identity and its authenticity against an authoritative source.

  As part of ongoing assurance, processes should require the holder to reaffirm their identity, reinterrogating and reestablishing its ownership as part of a continuous assurance journey. This process can occur through a variety of mechanisms as step-ups throughout the lifecycle of the credential. Examples of this can include an ongoing use of one-time-passwords (OTPs) on an established frequency - to ensure the user remains the holder; reauthorization of the credential using document authentication, biometrics, and liveness checks on an annual/biannual basis – research shows biometrics should be refreshed at least this frequently as part of best practices, amongst others.

  As a verifier, LNRS recommends CMS consider either only accepting credentials that have gone through these upfront and ongoing assurance processes, or that CMS leverage and recommend separate step-up processes when accepting verifiable credentials that fail to meet these standard requirements.

When each of the above steps is followed during issuance and as part of ongoing assurance, it provides other agency verifiers confidence in the credential they're accepting – that it's accurate to the identity and bound to the appropriate holder who is currently before them presenting it. Issuers should be encouraged to be transparent about their vendors and processes so verifiers can decide how trustworthy they wish to treat their credentials as they receive them.

- **Privacy:** Government-issued verifiable credentials offer substantial consumer privacy benefits by enabling individuals to control their personal information with precision and confidence. Unlike traditional paper-based records, these credentials support a variety of disclosure options, allowing users to share only the specific data required for a given interaction while keeping the rest private or enabling a verifier to request specific information or claims for their use case. This minimizes unnecessary exposure of sensitive information such as Personally Identifiable Information (PII) or Protected Health Information (PHI). Additionally, digital wallets empower users to store and manage their credentials securely, granting access only with explicit consent. Trust registries further enhance privacy by helping users verify the legitimacy of relying parties before sharing any data, thereby reducing the risk of phishing and unauthorized access. These privacy-preserving features collectively ensure that digital interactions remain secure, user-centric, and compliant with modern data protection standards.
- **Holder-Owned and Driven**: Accepting credentials from a holder-centric model empowers patients and beneficiaries to control their data and provide credentials at their discretion as proof to verifiers—such as healthcare providers or payers—without relying on constant connectivity to an issuing source. By accepting government-issued verifiable credentials, healthcare organizations can enhance patient privacy through technologies that support data minimization, anonymization, and consent-based sharing. At the same time, verifiers can maintain confidence in the authenticity of these credentials, as the underlying Public Key Infrastructure (PKI) ensures they are backed by authoritative government sources and cryptographically signed to prevent tampering. This approach not only strengthens trust but also supports more secure, efficient, and patient-centered digital interactions across the healthcare ecosystem.
- **Security:** Government-issued verifiable credentials offer significant security advantages over traditional paper-based documents. By leveraging cryptographic techniques such as digital signatures, these credentials ensure both the authenticity and integrity of the information they contain, making them resistant to tampering and forgery. Unlike static paper records, verifiable credentials can be dynamically revoked or refreshed, allowing issuers to respond swiftly to changes in status or potential compromises. They can also be bound to specific individuals and devices, reducing the risk of impersonation and unauthorized use. Furthermore, digital verification processes eliminate the need for manual inspection or specialized training, enabling secure, scalable, and automated validation across multiple channels. These features collectively enhance trust and reduce vulnerabilities in identity verification workflows.

Government-issued verifiable credentials represent a new wave of digital identity, however some challenges exist which should be raised and can be navigated though best practice approaches.

## *Challenges:*

- **Inconsistent processes:** A key challenge in the adoption of verifiable credentials is the variation in how agencies issue and verify them—particularly in sectors like healthcare, where accuracy, privacy, and trust are paramount. When processes for identity assessment, claims proofing, and credential validation differ across jurisdictions or systems, it can lead to inconsistent levels of assurance and complicate the ability of healthcare providers to confidently rely on presented credentials. This inconsistency can impact everything from patient onboarding to access to critical services. LNRS helps healthcare agencies overcome this challenge through solutions that leverage standards driven best-practices. These incorporate dynamic risk assessments, cryptographic binding, and interoperable protocols—ensuring that identities and the resulting credentials are issued and validated with uniform rigor. For healthcare organizations, this means greater confidence when they are presented and enhanced patient trust during digital interactions.
- **Adhering to Interoperable Standards:** A major challenge in the adoption of verifiable credentials arises when agencies fail to adhere to interoperable standards. Without a unified approach, credentials issued by one jurisdiction or system may not be recognized or accepted by others, leading to fragmented user experiences and diminished trust among verifiers. This lack of compatibility can hinder the seamless exchange of information, increase operational complexity, and discourage both issuers and relying parties from fully embracing digital solutions. LNRS helps agencies overcome this challenge by offering standards-aligned, interoperable solutions that are built on trusted and globally recognized frameworks. Our solutions ensure cryptographic security, machine-verifiability, and cross-platform functionality for cohesive assurance across digital trust ecosystems.

## *LNRS Capabilities:*

- LNRS supports the full spectrum of CMS' needs from Identity verification, authentication, and assurance of the Beneficiaries/Patients, Providers, and Verifiers to digital trust services and verifiable credentials offerings that adhere to industry standards for issuance and presentations. We are fully committed to supporting agencies on this next wave of digital identity as government-issuers and verifiers work to establish more secure processes across their interactive workflows.
- While verifiable digital credentials and the concepts we present in our response do not make the connection to provider data, the dynamic nature of provider data and efforts to make access to accurate, reliable and current provider data could converge as use cases are identified. This convergence could help those patients leveraging digital tools integrated into the ecosystem more efficient and effective at making care and coverage decisions. The LNRS referential provider database maintains over 160 million data records linked to over 12 million provider profiles. We leverage 3,000+ different sources, 450+ routine feeds from state license boards, daily updates from the Drug Enforcement Agency and data captured from claims data and other proprietary sources. Approximately 80% of all prescriptions in the U.S. leverage our provider data for verification before the prescriptions are filled, demonstrating our ability to address CMS needs on a nationwide scale.

# Question-specific responses

**PC-14. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 credentialing service providers (CSP)):**

**a. What are the challenges today in getting patients/caregivers to sign up and use digital identity credentials?**

Patients and caregivers may hesitate to use digital identity credentials due to concerns about data privacy and security. They want control over their information, when and how it is presented and assurance that it won't be leaked or breached. Additionally, they are wary of having their information in multiple systems and technologies that may not be reliable or secure over time.

**b. What could be the benefits to patients/caregivers if digital identity credentials were more widely used?**

Government-issued verifiable credentials offer privacy-preserving technology and user controls, allowing patients to manage their identity securely. These credentials, backed by consistent standards, enhance confidence and authenticity. Wider use in healthcare can streamline processes, reduce personal information exposure, and provide a portable health record for patients.

**c. What are the potential downsides?**

The effectiveness of CMS standards in the health ecosystem depends on their adoption and consistency. Centralized approaches may limit effectiveness for patients and providers. Inconsistent identity verification among issuers can lead to distrust in credentials. The downsides vary based on CMS's governance and processes.

**e. What role should CMS/payers, providers, and app developers have in driving adoption?**

CMS can adopt several roles within the digital identity healthcare ecosystem:
**Establish Standards:** Provide guidance and best practices for digital credentials, including identity verification, provider screening, credentialing processes, and health information and exchange standardization.
**Trusted Issuer:** CMS can facilitate trusted variable credentials for use within the health ecosystem.

- **Providers:** CMS can provide government-issued verifiable provider credentials and standardize these credentials for individual practitioners, provider organizations, and other healthcare entities as we move into an ever-increasing era of digital issuance. During provider credential issuance there is a high need for provider screening as well as an opportunity to standardize future provider types, such as atypical providers who facilitate critical patient services.
- **Patients:** CMS can provide government-issued verifiable patient credentials for participants who qualify for its various programs (Medicaid, CHIP, Medicare Parts A-D, etc.).

- **Trusted Verifier:** CMS can be a trusted verifier for government-issued verifiable credentials. CMS can accept other government-issued verifiable credentials as proof-of-identity with additional identity step-ups as required.
- CMS can accept and establish clear verification requirements for the verifiable credentials.
- CMS can establish a process for emerging verifiable credentials to become recognized and accepted.
- CMS can serve as the trust registry for Provider credential transactions, ensuring government and healthcare entities undergo proper identity and business assurance, credentialing, and act within the boundaries of approved use cases, creating public keys for accredited issuers and verifiers.
- CMS can enforce a common set of rules every participant and service must be verified against.
- CMS can ensure credentials are standards-based, interoperable, and verifiable while ensuring data sovereignty

**f. How can CMS encourage patients to get digital identity credentials?**

To encourage patients to adopt digital identity credentials, CMS can promote best practices that include:

- Standard identity assurance approaches to ensure information is only presented to the intended holder
- Interoperable technologies that follow leading standards
- Privacy preserving approaches that advocate for data minimization and user-permissioned disclosure
- Holder-centric approaches that provide holders their health information for local storage and portable use
- Standard verification approaches that require public key validation, status checks for revocation or expiration, and binding of the credential to the appropriate holder
- Credential management requirements to ensure ongoing validity as credentials, like people, are dynamic.

## 3. Digital Identity

**PR-9. How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?**

Holder-driven approaches are becoming more common, and login events can be more secure with government-issued verifiable credentials and identity step-ups. Using multiple government-issued credentials can provide higher identity assurance. These methods may eventually make proprietary logins and CSP approaches inadequate.

**a. What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider?**

Government-issued credentials can standardize logins when presented by the holder. CMS could create verifiable credentials for healthcare use cases to ease adoption across providers or incentivize providers to accept credentials via ISO/IEC 18013-5:2021 and OpenID4VP ISO/IEC TS 18013-7:2024 formats.

**b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?**

Patient privacy and convenience can coexist when information is provided to the patient for their use and disclosure through privacy-preserving verifiable credential workflows. These credentials can expedite interactions and initiate additional health inquiries as needed. For example, a verifiable credential can be provided during an office visit with relevant high-level claim information for easy transmission. This would be like a patient providing their identity document and a short medical history form, but instead of using pen, paper, and ID cards, the information can be seamlessly transmitted by the patient to the provider. The patient can also grant consent for additional medical history to be requested by the practitioner from other providers. As new records become available, the patient can capture these claims as part of their credentials.

**PR-10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs):**

**a. What are the challenges and benefits for providers?**

Government-issued verifiable credentials are emerging as a reliable option for identity verification in the U.S. when coupled with best practice identity verification and assurance processes. They offer consistent standards, evolving practices, and stable technology, making them suitable for various verifier use cases. Further, these credentials provide an opportunity for relevant information (claims) to be conveyed at the patient/holder’s request for a variety of verifier use case needs.

**b. How would requiring their use improve access to health information?**

Using verifiable credentials can help healthcare organizations quickly verify a patient's identity, understand their household composition, insurance coverage, and relevant health information. Binding this information to the patient's wallet ensures it remains confidential, private, portable, and available for various use cases, including emergencies.

**c. What are the potential downsides?**

Relying on non-government issued credentials poses a significant risk due to the lack of public trust and stability. However, public-private partnerships can leverage private-sector technologies to deliver these services, benefiting from best-of-breed offerings and best practices.

**d. What impact would mandatory credentials have on a nationwide provider directory?**

Verifiable credentials for providers could enable CMS to establish a national provider directory, reinventing NPI issuance, provider screening, and verification processes. This approach should leverage identity assurance provider screening journeys similar to those used for issuing verifiable credentials to citizens. It could standardize atypical providers at state and national levels.

**e. How could digital identity implementation improve provider data flow?**

Provider data flows could rely on verifiable credential presentations to confirm identity ownership. These credentials could establish a unique identifier for a Master Person Index (MPI) and reaffirm it during subsequent visits. Similarly, government-issued provider verifiable credentials could streamline provider network interactions, care team coordination, and referral recommendations.

**f. Would combining FHIR addresses and identity improve data flow?**

Verifier requests for additional health information can trigger FHIR services if the patient/holder wishes to provide it. This can occur through various workflows, including presentation events.

**PR-11. How could members of trust communities (for example, QHINs, participants and subparticipants in TEFCA, which requires Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs)) better support the goals of reduced provider and patient burden while also enhancing identity management and security?**

Trust communities can advocate for best practices and government-issued verifiable credentials for identity assurance. They can also ensure high-assurance gates and requirements are met. Verifiable credentials should only be relied on if they follow IAL2 or higher standards for digital issuance. Some current credentials, like certain mDLs, don't yet meet these standards for digital issuance, only physical issuance, to reduce user friction and foster adoption. This is an area for improvement that trust communities can help advance.

**PA-3. How can CMS encourage payers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins?**

*This is addressed in the answer to PR-9.*

**PA-4. What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?**

*This is addressed in the answer to PR-9.*

**TD-3. Regarding digital identity implementation:**

**a. What are the challenges and benefits?**

Identity implementations vary widely, but core components remain consistent across workflows. Automation and self-service approaches, supported by proper technology, can eliminate manual workflows. Best practices demand simplicity through seamless orchestration platforms for dynamic decision-making and superior identity assurance. Identity solutions should offer low-code integration and deploy industry-leading verification, authentication, biometric, and liveness capabilities to prevent fraud and ensure compliance. Healthcare entities shouldn't sacrifice services or security for reliable identity assurance.

Depending on the type of identity solution being implemented, this can be categorized into:
Identity verification, authentication, and assurance integrations: These are typical during issuance and ongoing assurance events. These are often deployed for account creation, login, or verifiable credential NIST 800-63-3 IAL2 identity journeys. A simple single API can provide these services with pass/fail decisioning. Component based services should also be available for atypical workflows and/or agency users.

Call-center or otherwise remotely deployed identity assurance integrations: These are typical during assisted step-up journeys or when users lack access to typical technology.

**b. How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?**

Relying on federated identity approaches can place agencies and verifiers at risk. Security and trust are foundational for government services, but identity verification is increasingly challenging. AI has accelerated fraud, with Deepfakes used in targeted attacks. Federated approaches, trusted by many, are prime targets and create potential risks if relied upon exclusively.

**c. What impact would mandatory use of the OpenID Connect identity protocol have?**

While OpenID Connect is useful for authentication, it relies on centralized identity providers, limiting user control and privacy. CMS should evaluate OpenID for Verifiable Credentials (OID4VC) for a more decentralized and secure approach, allowing individuals to manage and present their credentials independently.